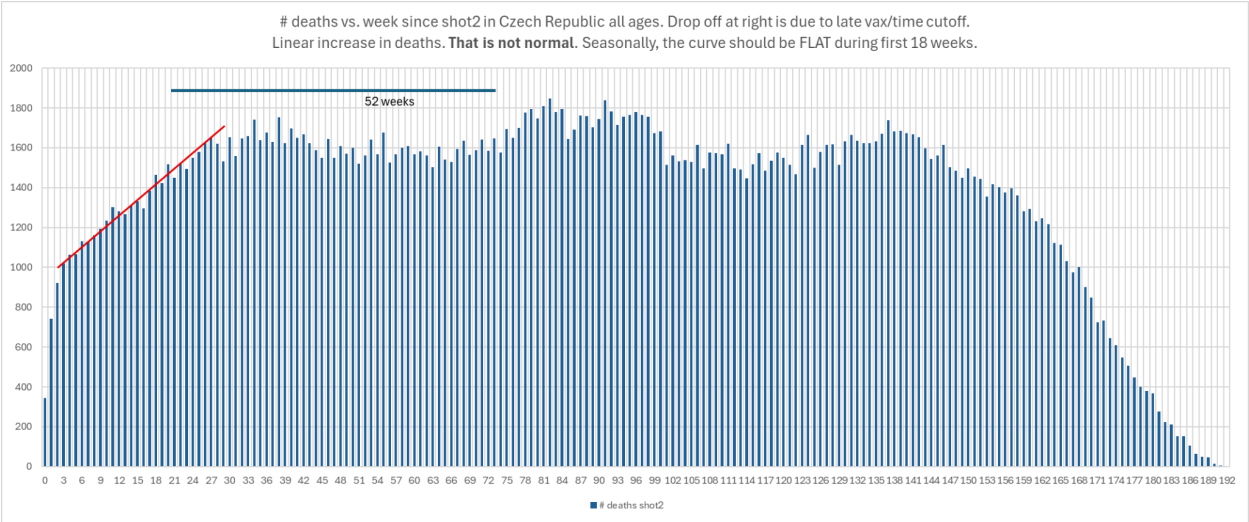
This chart is deaths vs. time sinceshot 2 COVID shots (all ages) from deaths.xlsx in the Czech repo:



The first two weeks are pathologically low. That's dynamic HVE.

Hungary

i. The curvature (instantaneous hazard value) of the unvaccinated cohort is constant over time. This validates the KCOR assumption of constant hazards (that can be slope adjusted). The vaccinated curves have opposite curvature (and higher than expected if no selection bias) exactly as expected.
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A 3x h(t) can have massive implications

Suppose only the most frail can die from COVID.

The unvaxxed have a heterogeneous mix of people with, on average, a 3x higher mortality as we learned earlier.

So it makes sense (though hard to prove with the data we have) that there is likely way more diversity of mortality in the unvaccinated cohort at both ends of the spectrum.

So given a hypothesis that only the most frail die from COVID, it implies that the unvaccinated might have 10x more people that qualify due to the heterogeneity.

So even if the vaccine did nothing, it would appear to have a miraculous effect because of this.

Japan had higher shots per person, an older population, so had a “perfect storm” impact.

Note that By March 23, 2022, about **95% of the eligible adult population (16+) in Australia was fully vaccinated** with at least the primary 2-dose series. So how their COVID excess mortality could climb so consistently and precipitously since the shots began is hard to explain.

Their official stance is that the excess deaths were mostly COVID deaths (which is interesting since the vaccines were supposed to minimize COVID deaths), but as you can see that after these levelled off, the excess deaths still climbed at the same rate!

Why you need slope normalization for cohorts age >60

KCOR features slope normalization for cohorts, something you rarely see in retrospective comparison studies. This is because KCOR was designed for vaccine data where there is a heavy HVE effect which cause the unvaccinated cohort (higher mortality and frailty) to have an h(t) function that doesn’t have the normal log-slope of 8.5% per annum as a normal cohort.

This is caused by heterogeneity and depletion effects and is noticeable for cohorts over 60 years old. Generally, if any cohort of sufficient size is “pathological” (i.e, not the standard annual slope), all the cohorts are affected.

Empirically we find slope normalization with a constant is effective below age 90. Above 90, using a constant for the log-slope is too crude.

See KCOR_analysis:log slope test (3 tabs) for details.

Enrollment date matters

1. Right after rollout start where many got dose 1 and few got dose 2: dose 1 and 2 will merge but capture
2. In the middle after half the people got dose 2: We'll: catch early vax harm and preserve differential between Dose 1 and Dose 2 cohorts
3. Too late: We'll miss the vax harm and both Dose 1 and 2 will look safe.

This is why we looked at various enrollment dates.

Abbreviations

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Add

2. log-slope tests in KCOR_analysis.shows slope valid <90
3. Show full dataset validation by 8.5% slope.
4. Can uses all data if after 1930
5. Add the CI computation to the spreadsheet
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7. Cum full adj shows same slope at start and end… dataset is stunningly valid with mortality of 8.5%/yr as expected. No data gaps
8. Add week 20 enrollment date! Check for sensitivity
9. US Medicare COVID shot HVE
10. Let's be conservative on HVE and baseline length

11. Booster is where you can tell it isn't HVE driving this using 3 v 0 compared to 2 v 0. Stable reference point. Primary shots too confounded: was dose 2 h(t) rising due to HVE or the vaccine? Too hard to tell since h(t) of dose 0 lowering with COVID wave ending! With booster have a stable reference.
12. Section fore KCOR results
13. HVE
    a. Looks like HVE for sure
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14. Static HVE is 3x
    a. See CMR: 2021-24 3x static HVE

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2. COVID was NOT a proportional hazard. Proof. So KCOR results are optimistic favoring the vaccine.
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effects, you must slope-normalize all cohorts before comparison which is exactly what KCOR does. Otherwise, the comparison will appear to make the vaccine look more harmful over time than it really is (since the vaxxed mortality rate increases over time at a higher rate than normal, but the unvaxxed mortality rate will actually decrease over time). You can see this visually in the KM plots in the Hungary study.

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```
Dose combination: 1 vs 0 [2021_24]
--------------------------------------------------
          YoB | KCOR [95% CI] | KCOR_ns
--------------------------------------------------
  ASMR (direct)   |   1.0520 [1.036, 1.068] | -
  1920            |   0.9001 [0.846, 0.958] | 0.9459
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  1990            |   2.5299 [2.010, 3.184] | 3.9998
  2000            |   1.3448 [0.884, 2.046] | 1.0927
```

```
Dose combination: 2 vs 0 [2021_24]
-----------------------------------------------------
                YoB | KCOR [95% CI] | KCOR_ns
-----------------------------------------------------
  ASMR (direct)    |   1.1876 [1.174, 1.202] | -
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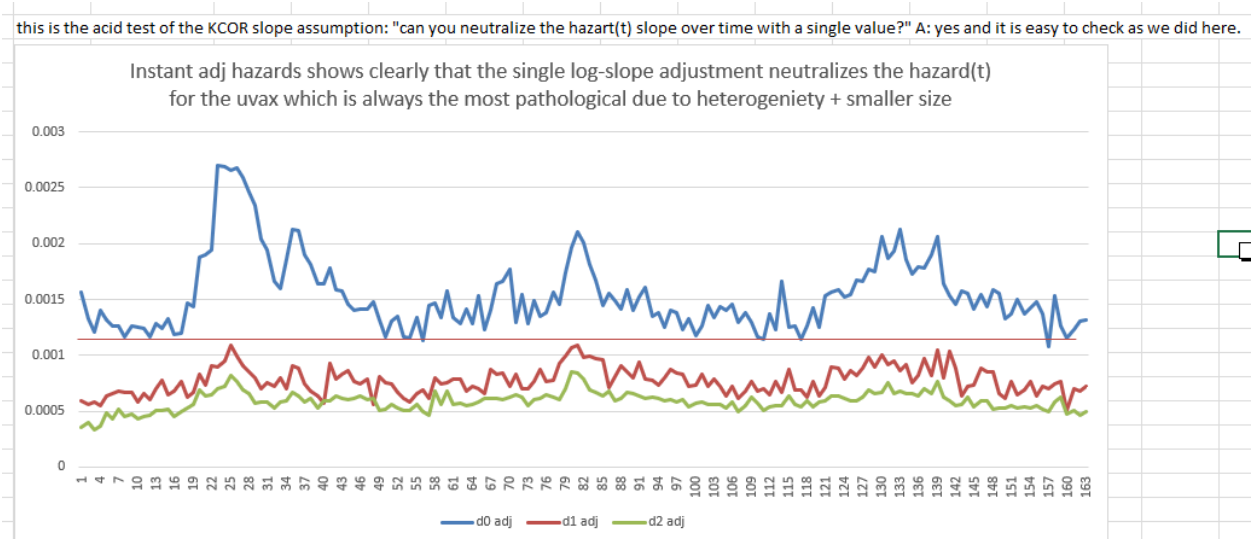
```

```
Dose combination: 3 vs 0 [2022_06]
-----------------------------------------------------
                YoB | KCOR [95% CI] | KCOR_ns
-----------------------------------------------------
  ASMR (direct)    |   1.4926 [1.467, 1.518] | -
  1920             |   1.2074 [1.141, 1.278] | 1.4031
  1930             |   1.4701 [1.422, 1.519] | 1.6403
  1940             |   1.5942 [1.544, 1.646] | 1.8146
  1950             |   1.5825 [1.521, 1.646] | 1.7762
  1960             |   1.2821 [1.206, 1.363] | 1.4229
  1970             |   1.3494 [1.232, 1.478] | 1.6409
  1980             |   0.9958 [0.848, 1.169] | 1.1953
  1990             |   1.2055 [0.929, 1.565] | 1.4701
  2000             |   0.8977 [0.577, 1.397] | 1.0949
```

16. See the KCOR section below for the KCOR(t) plot for these cases above (with CI bands).
17. KCOR is the best way to assess net harm/benefit. KCOR's math has been independently validated many times. KCOR relies on the key assumption that you can slope normalize hazard(t) over the region of interest using a single log-slope value. I've found that this assumption is unlikely to be violated in real-world cohorts created with selection bias simply because the cohorts don't change composition that rapidly. It is

trivial to validate the slope assumption by inspection. It **passes the key assumption with flying colors:** see the 194x cohort where **we can draw a flat line through the adjusted instantaneous hazards of the unvaccinated. The 80-90 year old unvaccinated cohort is equally impressive** (source KCOR_sample_computation and KCOR_analysis):



18. If there is an error in KCOR, SW needs to tell us where it is. If there is a better method, SW needs to produce it.
19. If the COVID vaccine was net life saving, two doses should save more lives than two. KCOR shows that the 2 dose curve is always above the 1 dose curve. This means the vaccine was net harmful.
20. Detecting the COVID vaccine net mortality impact from observational studies is extremely challenging due to overlapping possible causes of deaths (seasonality, COVID, etc).

    In short, it was easy to dismiss the COVID vax deaths because they could be lumped into the COVID deaths. In the US, there was clearly a population mortality increase. But was it 100% COVID and 0% vaccine?

    It was easy to detect COVID deaths because there was a test and they came in waves fast and hard. You are rewarded for finding a COVID death and fired for finding a vaccine death. So these “died suddenly” deaths were smoking guns.

    Japan is the confirmation. We have record level data and see the same divergence of vaccinated mortality post shot. And we see the cumulative excess deaths go almost straight up. **When have you seen that before?**

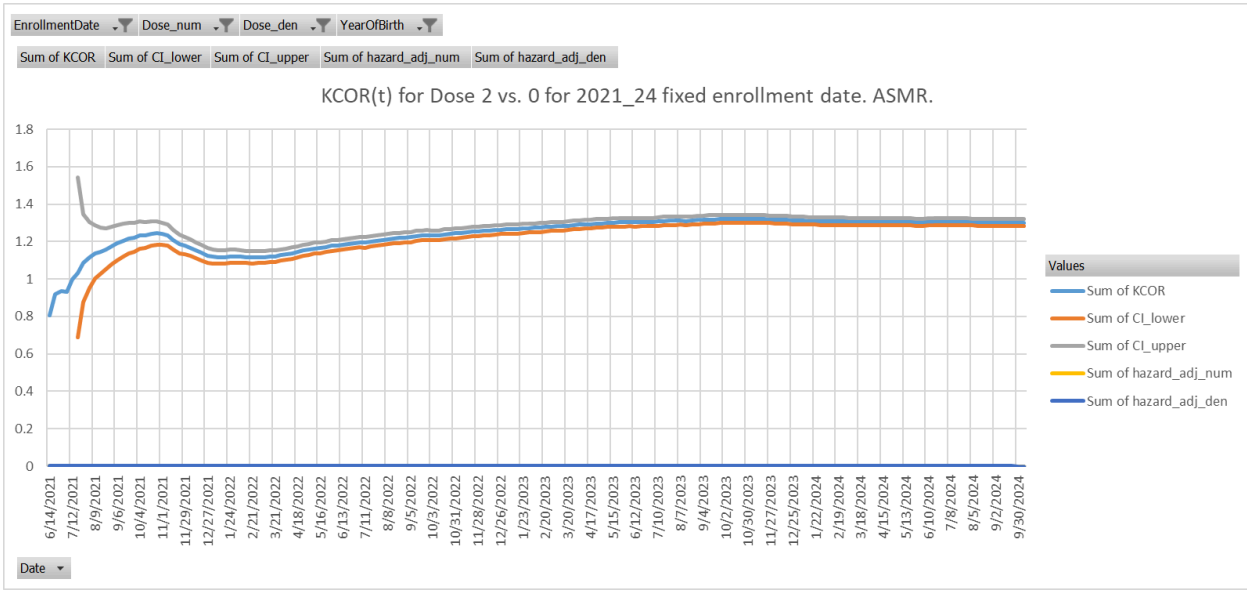
    Detecting vaccine deaths is much more difficult, requiring sensitive methods that look at v vs. u cohorts such as KCOR. I don’t know of anything better.

Key points

1. Hypothesis

The COVID vax mortality benefit was a mirage; it was created by static HVE. The smoking gun is easy to see.

Vaccines increased ACM in those that took them for 6 months and then it plateaued. There is a rise in mortality for 6 months after people got the COVID shot and it then plateaus. This is inconsistent with a “safe vaccine” which would have no mortality rise. We can see this clearly in the KCOR curve for mid June 2021 enrollment, born in 1940. The line is always above baseline 1. There is an apparent COVID vaccine death benefit, but this is due to the mortality difference between the 1940 cohorts (they are the same age, but the mortality difference is over 3x (see KCOR_CMR_analysis). But even with the false benefit, there was a

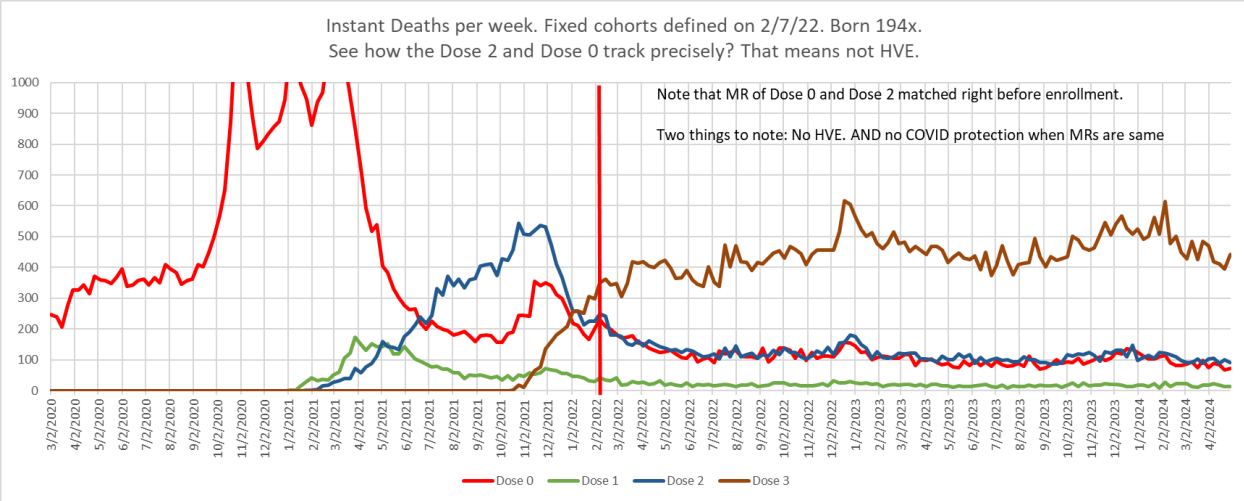


The rapid mortality increase post-vaccine cannot be explained by selection bias because it isn’t mirrored in the N-1 cohort. It is a real effect. Attempts to explain the mortality rise as caused by selection bias (dynamic HVE) is easily falsified.

To examine HVE, you must have fixed cohorts, look at death COUNTS, look at a narrow age range. So not ASMR and not time-varying.

It’s **easiest to falsify HVE using the booster shot enrollment date** for born 194x because all the deaths have to appear in Dose 2 if it is HVE. See how dose 2 and 0 exactly track the entire time? This kind of tracking is stunning!

It simply can't happen if Dose 2 is accepting massive deaths from Dose 3. (source: KCOR_CMR_analysis/booster HVE falsification):

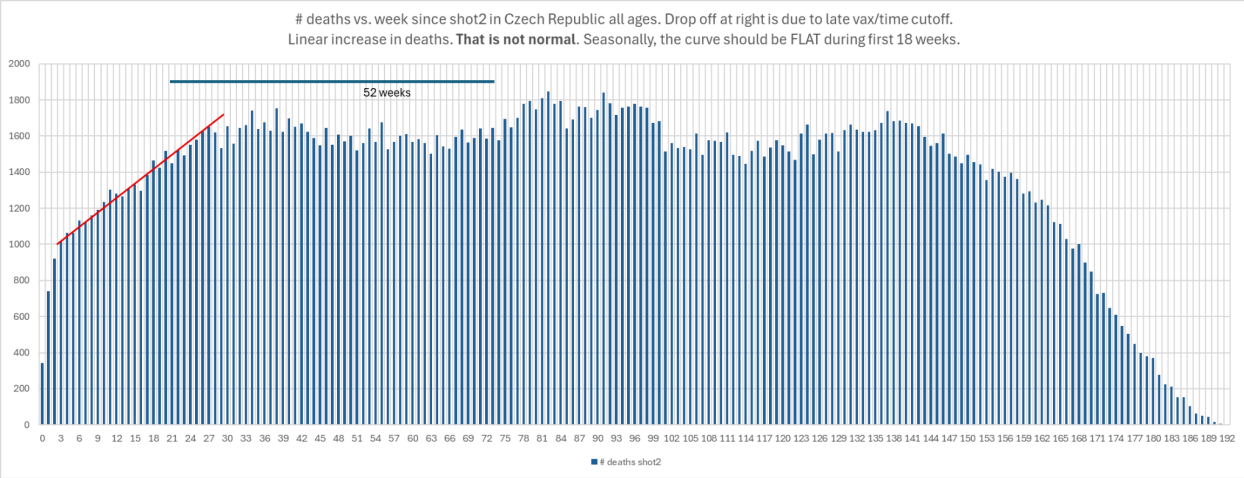


We can also falsify dynamic HVE using KCOR by comparing 3 v 0 and 2 v 0 for the boosters. They are not mirror images as they would be if the rise in deaths in Dose 3 were deaths left behind in the Dose 2 group. It shows Dose 2 had a flat slope compared to dose 0 for the booster enrollment (after Dose 2 mortality had increased and stabilized). So the mortality increase in the Dose 3 group was not a selection bias artifact. Remember: **these are all fixed cohorts**. So the only viable explanation is vaccine harm. So dose 2 vs 0 is flat because the harms happen within 6 months and flat after that. So it shows exactly what we'd expect to show. The curve below is ASMR, but the 1940 decade curve looks the same.

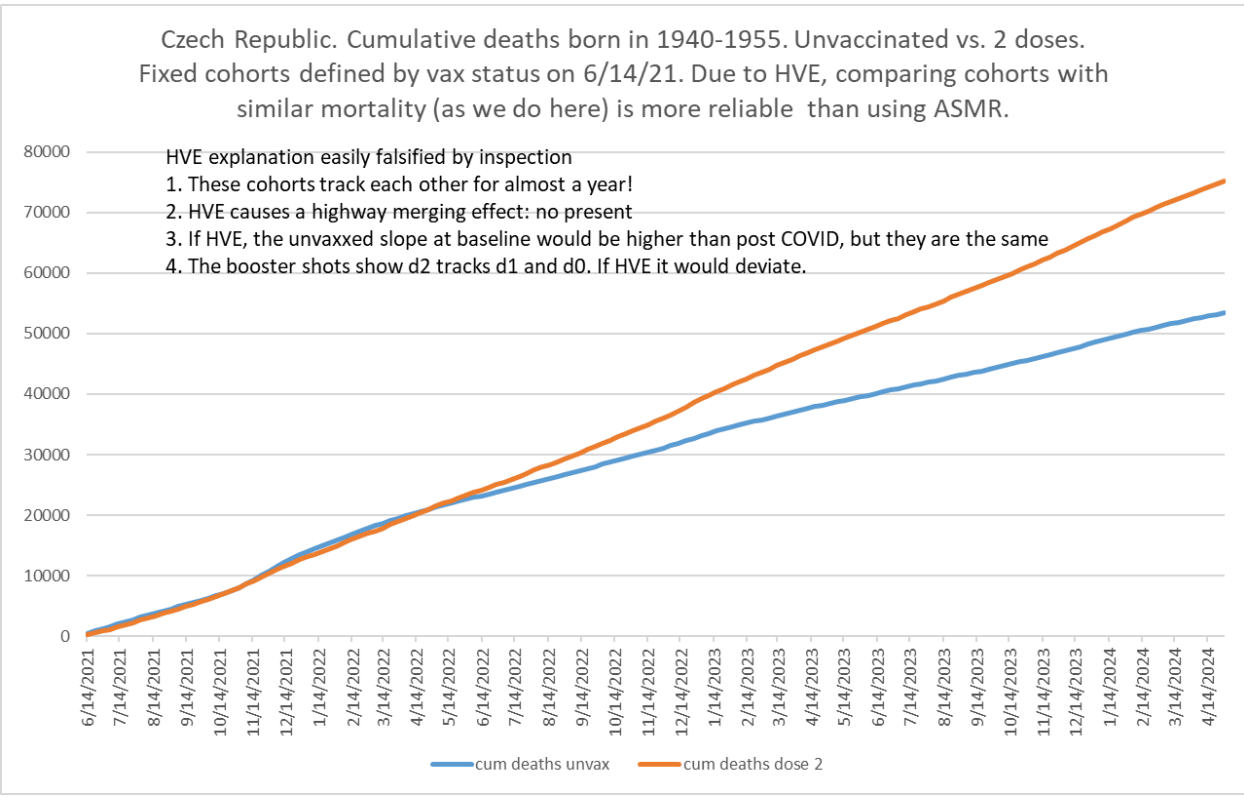
So the rise in deaths wasn't caused by bias transfer of deaths to Dose 2.

Time series should be flat for a safe vaccine. It isn't. The event time series plots for the COVID vaccines show a linear (not exponential) rise, confirming the rise in death is correlated with the shot timing. The plots are the same shape for all age groups (source: deaths.xlsx). This wasn't seasonality because seasonality is an elderly effect and happens only near December. Also, if it was seasonal, it

would repeat at the same levels.



If you match up cohorts on deaths per week, y**ou find the cohorts diverge after the booster shots are given even though we are using fixed cohorts and following the SAME cohort longitudinally**. You cannot have a longitudinal analysis of a fixed cohort who are given a life-saving intervention have mortality that goes the wrong way (Source: KCOR_CMR_analysis):



Harm >> any benefit. KCOR shows that the sustained mortality rate increase was significantly larger than the time limited mortality rate differential during peak COVID→ harm > benefit. This is easiest to see by the value of KCOR(t) at the end of 2022.

The negative log-slope for elderly unvaccinated is normal and due to cohort heterogeneity. You must adjust for this to compare cohorts. This is a standard step in KCOR. No other epidemiological method does this. Yet, without that step, your comparisons are significantly wrong. Most epidemiologists are clueless about this.

2. My 3 cited papers and what they show:

   Qatar study: They did the most extreme 1:1 matching ever done in a study. They found two things: 1) you can't adjust away HVE using covariates and 2) despite a huge HVE bias reducing death, aHR after 6 months was >1.

   "In the first 6 months of follow-up in the two-dose analysis, the aHR was **0.35** (95% CI: 0.27–0.46); however, the combined analysis of all subsequent periods showed an aHR of **1.52** (95% CI: 1.19–1.94). In the first 6 months of follow-up in the three-dose analysis, the aHR was **0.31** (95% CI: 0.20–0.50); however, the combined analysis of all subsequent periods showed an aHR of **1.37** (95% CI: 1.02–1.85)."

   So even with a huge HVE advantage, the aHR went from extremely protective to >1 after 6 months showing vaccine harm, an incompensable HVE effect, and **confirming what KCOR found** about significant mortality increase by 6 months.

   Denmark study: They also did anal 1:1 matching and effectively said "we give up. HVE so big and hard to neutralize, we need RCTs. Traditional epi analysis methods don't work." Here is their Conclusion:

   Conclusion: Our findings indicate that observational studies of SARS-CoV-2 vaccine effectiveness may be **subject to substantial confounding**. Therefore, **randomized trials are essential** to establish vaccine efficacy after the emergence of new SARS-CoV-2 variants and the rollout of multiple booster vaccines.

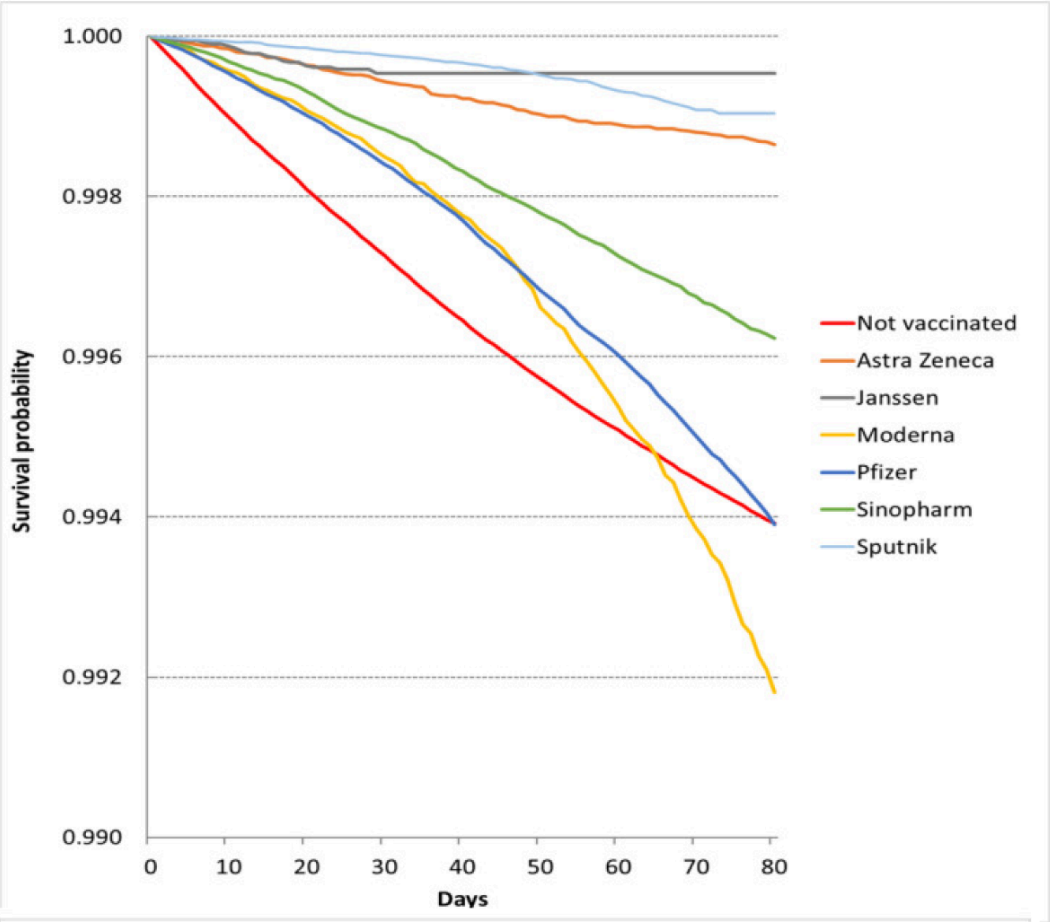
   But what they should have highlighted, but chose to avoid, is the negative control failed for the vaccinated cohort: 3.05 (95% CI: 2.24–4.14) for malignant melanoma. In short, vaccination negatively impacts your immune system so we have biologic plausibility here. This is a smoking gun of immune system harm.

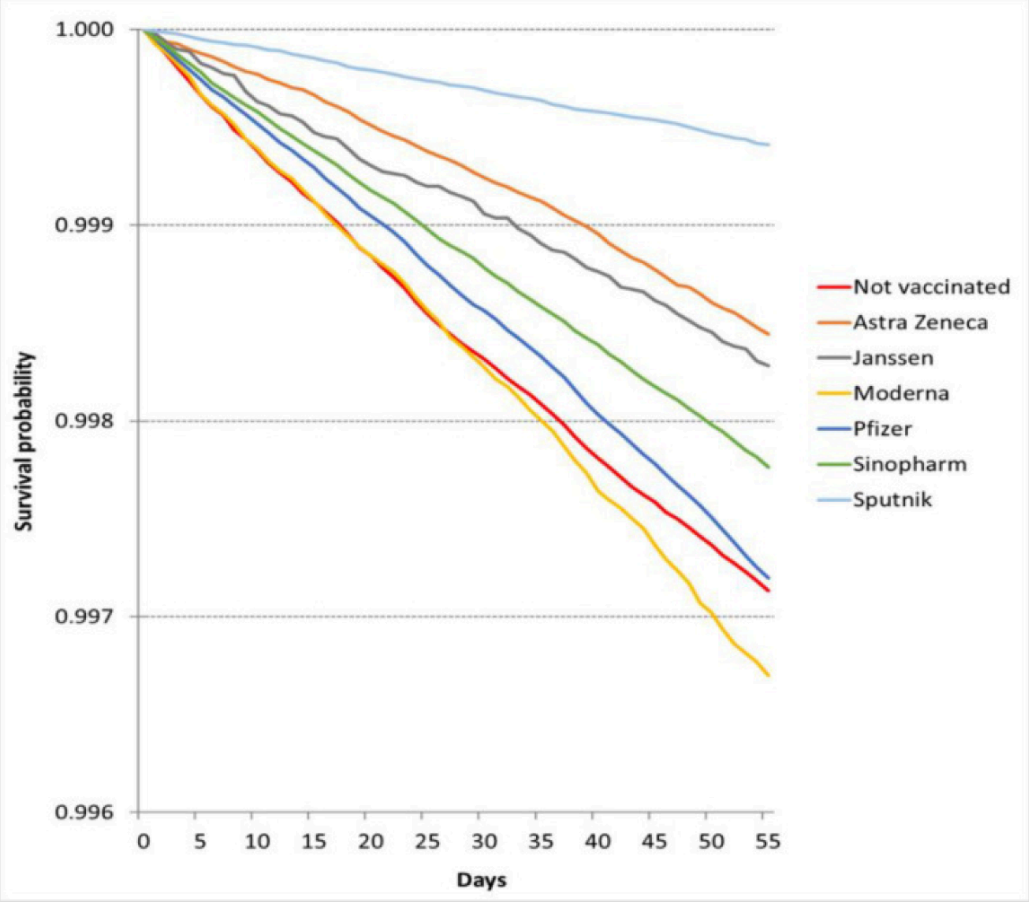
   When a study like this shows "favorable" noise in non-biological outcomes (like trauma deaths) but *unfavorable biological outcomes* (like cancer), anyone with intellectual honesty should admit that the *supposed safety reassurance is actually the opposite:* ***it's a huge red flag.***

   So this study confirms harms to the immune system and **also, like the Qatar study, throws all other COVID vaccine observational studies under the bus**

because you can't compensate for the HVE effect. This is why KCOR is so important because it works where other epi methods fail. It is the only method using slope normalization, for example. No other epi method does this; KCOR is unique to vaccinated mortality studies. KCOR is a general purpose epi method that is particularly adept at dealing with vaccine net mortality assessments.

Hungary study: Fig 1 and 2 are the smoking guns. These are KM plots. They reveal 3 huge red flags:


1.000
0.998
0.996
0.994
0.992
0.990
Survival probability
0
10
20
30
40
50
60
70
80
Days
Not vaccinated
Astra Zeneca
Janssen
Moderna
Pfizer
Sinopharm
Sputnik


1.000
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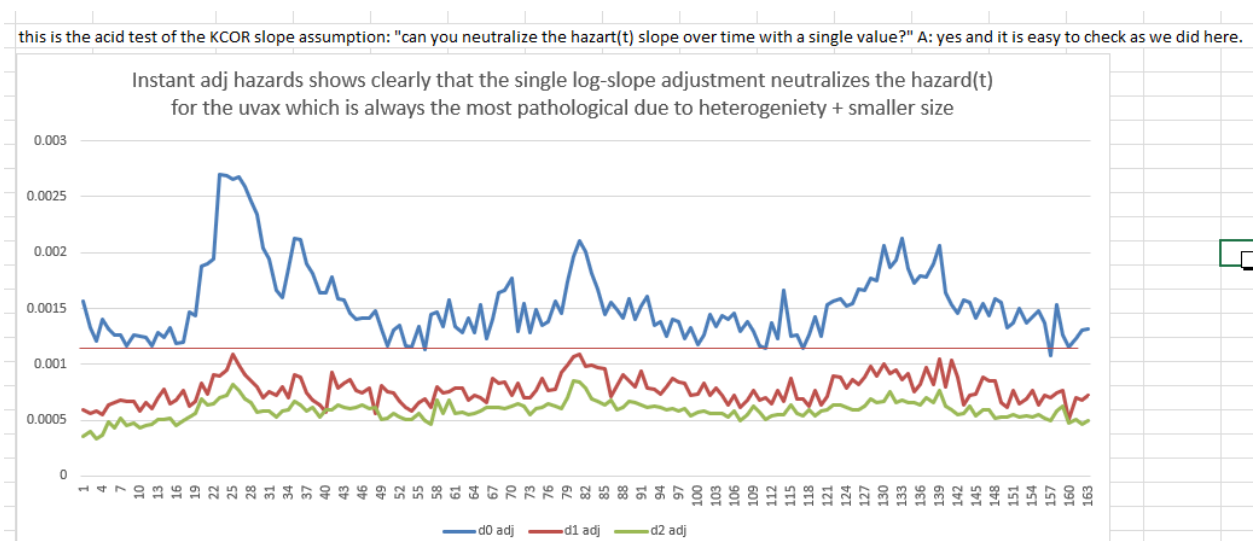
-------------------------------------------------
            YoB | KCOR [95% CI] | KCOR_ns
-------------------------------------------------
  ASMR (direct)   |   1.4926 [1.467, 1.518] | -
  1920            |   1.2074 [1.141, 1.278] | 1.4031
  1930            |   1.4701 [1.422, 1.519] | 1.6403
  1940            |   1.5942 [1.544, 1.646] | 1.8146
  1950            |   1.5825 [1.521, 1.646] | 1.7762
  1960            |   1.2821 [1.206, 1.363] | 1.4229
  1970            |   1.3494 [1.232, 1.478] | 1.6409
  1980            |   0.9958 [0.848, 1.169] | 1.1953
  1990            |   1.2055 [0.929, 1.565] | 1.4701
  2000            |   0.8977 [0.577, 1.397] | 1.0949
```

36. See the KCOR section below for the KCOR(t) plot for these cases above (with CI bands).
37. KCOR is the best way to assess net harm/benefit. KCOR's math has been independently validated many times. KCOR relies on the key assumption that you can slope normalize hazard(t) over the region of interest using a single log-slope value. I've found that this assumption is unlikely to be violated in real-world cohorts created with selection bias simply because the cohorts don't change composition that rapidly. It is trivial to validate the slope assumption by inspection. It **passes the key assumption with flying colors:** see the 194x cohort where **we can draw a flat line through the adjusted instantaneous hazards of the unvaccinated. The 80-90 year old unvaccinated cohort is equally impressive** (source KCOR_sample_computation and KCOR_analysis):



38. If there is an error in KCOR, SW needs to tell us where it is. If there is a better method, SW needs to produce it.

39. If the COVID vaccine was net life saving, two doses should save more lives than two. KCOR shows that the 2 dose curve is always above the 1 dose curve. This means the vaccine was net harmful.
40. Detecting the COVID vaccine net mortality impact from observational studies is extremely challenging due to overlapping possible causes of deaths (seasonality, COVID, etc).

    In short, it was easy to dismiss the COVID vax deaths because they could be lumped into the COVID deaths. In the US, there was clearly a population mortality increase. But was it 100% COVID and 0% vaccine?

    It was easy to detect COVID deaths because there was a test and they came in waves fast and hard. You are rewarded for finding a COVID death and fired for finding a vaccine death. So these “died suddenly” deaths were smoking guns.

    Japan is the confirmation. We have record level data and see the same divergence of vaccinated mortality post shot. And we see the cumulative excess deaths go almost straight up. **When have you seen that before?**

    Detecting vaccine deaths is much more difficult, requiring sensitive methods that look at v vs. u cohorts such as KCOR. I don’t know of anything better.

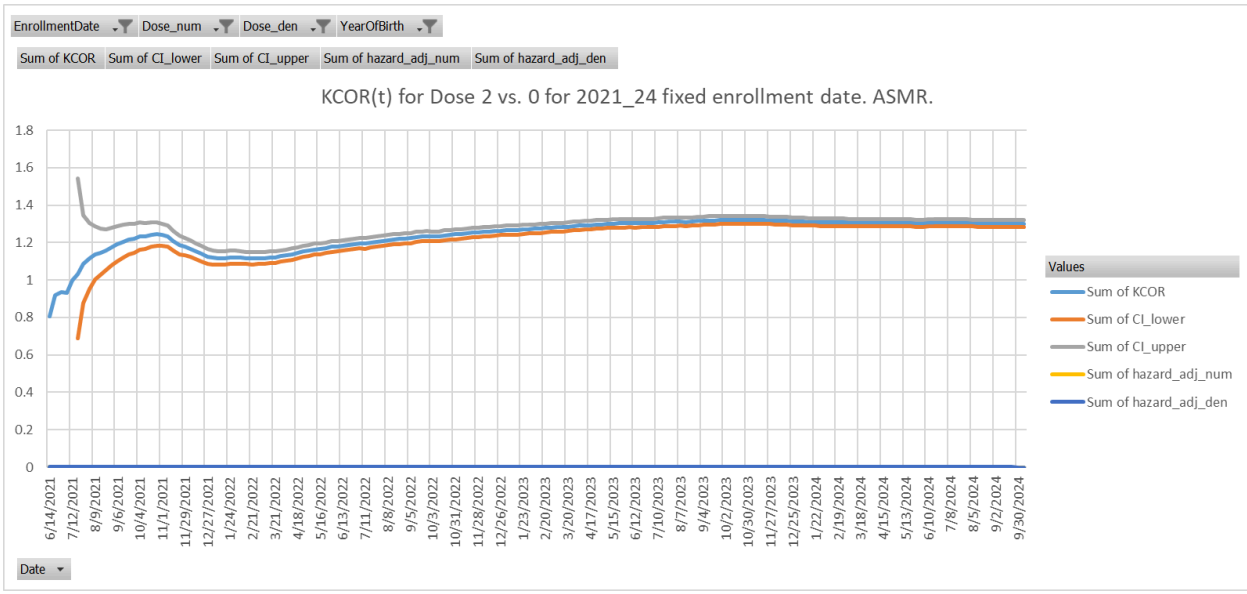
Key points

3. Hypothesis

    The COVID vax mortality benefit was a mirage; it was created by static HVE. The smoking gun is easy to see.

    Vaccines increased ACM in those that took them for 6 months and then it plateaued. There is a rise in mortality for 6 months after people got the COVID shot and it then plateaus. This is inconsistent with a “safe vaccine” which would have no mortality rise. We can see this clearly in the KCOR curve for mid June 2021 enrollment, born in 1940. The line is always above baseline 1. There is an apparent COVID vaccine death benefit, but this is due to the mortality difference between the 1940 cohorts (they are the same age, but the mortality difference is

over 3x (see KCOR_CMR_analysis). But even with the false benefit, there was a

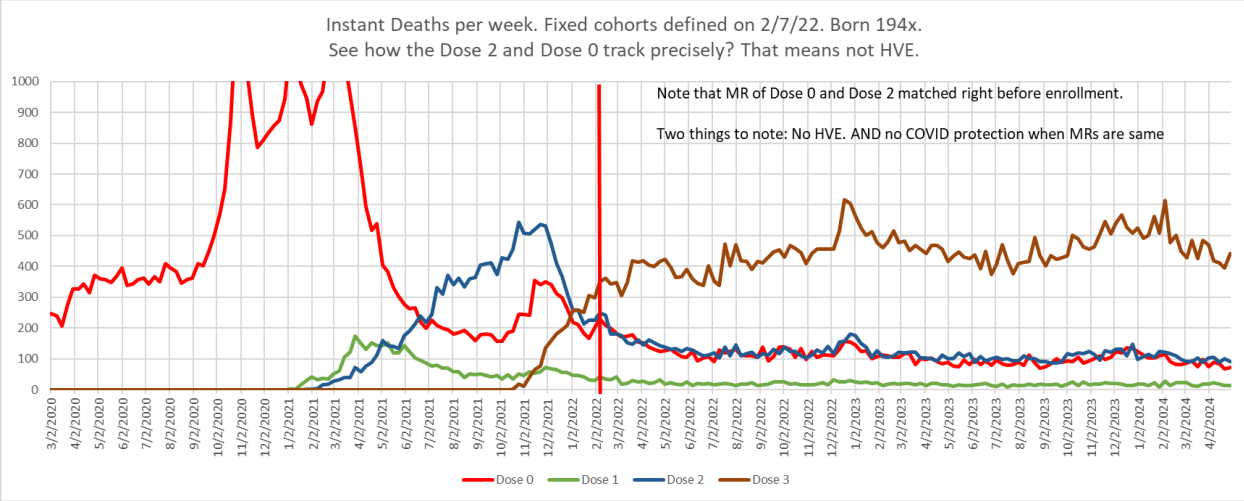


The rapid mortality increase post-vaccine cannot be explained by selection bias because it isn't mirrored in the N-1 cohort. It is a real effect. Attempts to explain the mortality rise as caused by selection bias (dynamic HVE) is easily falsified.

To examine HVE, you must have fixed cohorts, look at death COUNTS, look at a narrow age range. So not ASMR and not time-varying.

It's **easiest to falsify HVE using the booster shot enrollment date** for born 194x because all the deaths have to appear in Dose 2 if it is HVE. See how dose 2 and 0 exactly track the entire time? This kind of tracking is stunning!

It simply can't happen if Dose 2 is accepting massive deaths from Dose 3. (source: KCOR_CMR_analysis/booster HVE falsification):

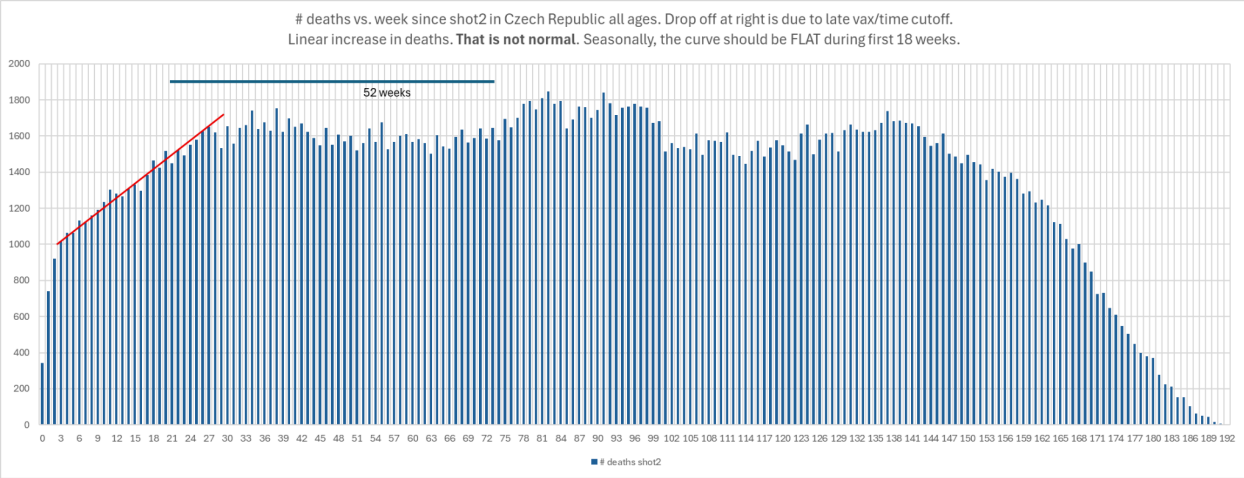


We can also falsify dynamic HVE using KCOR by comparing 3 v 0 and 2 v 0 for the boosters. They are not mirror images as they would be if the rise in deaths in Dose 3 were deaths left behind in the Dose 2 group. It shows Dose 2 had a flat slope compared to dose 0 for the booster enrollment (after Dose 2 mortality had increased and stabilized). So the mortality increase in the Dose 3 group was not a selection bias artifact. Remember: **these are all fixed cohorts**. So the only viable explanation is vaccine harm. So dose 2 vs 0 is flat because the harms happen within 6 months and flat after that. So it shows exactly what we'd expect to show. The curve below is ASMR, but the 1940 decade curve looks the same.

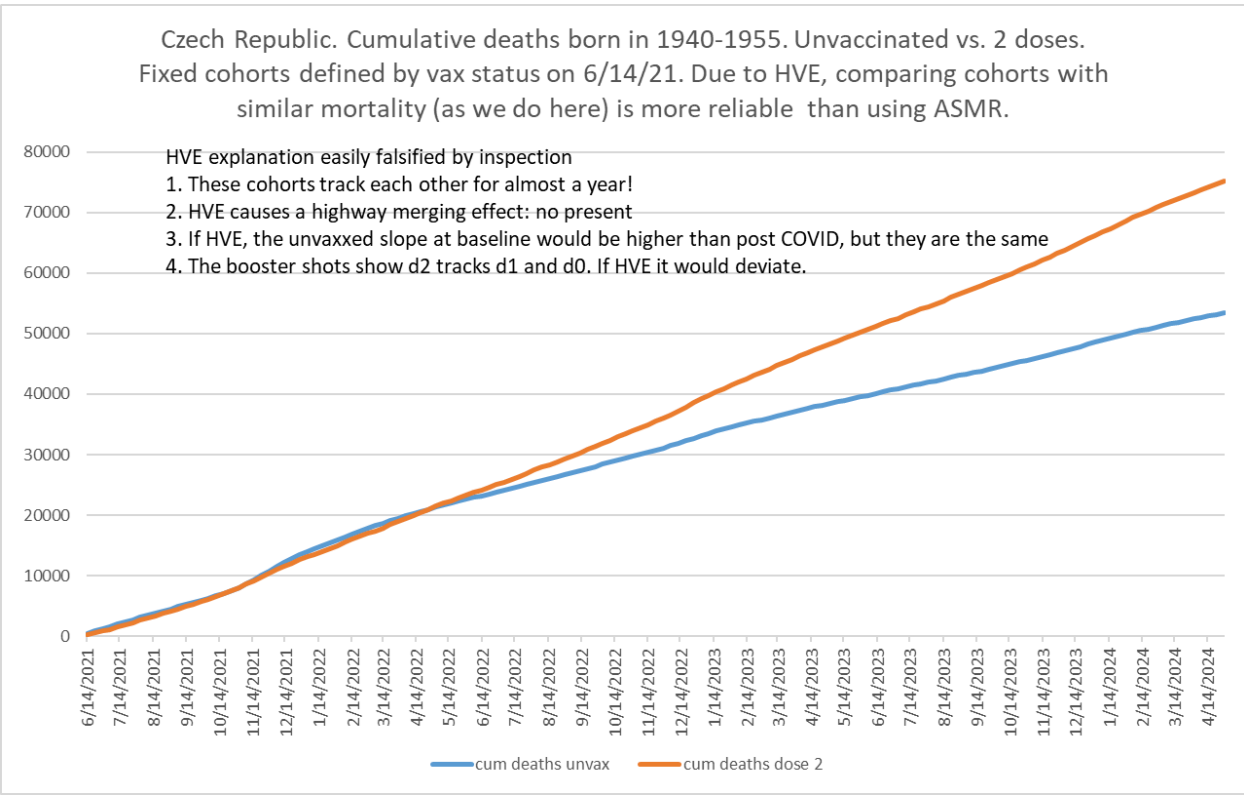
So the rise in deaths wasn't caused by bias transfer of deaths to Dose 2.

Time series should be flat for a safe vaccine. It isn't. The event time series plots for the COVID vaccines show a linear (not exponential) rise, confirming the rise in death is correlated with the shot timing. The plots are the same shape for all age groups (source: deaths.xlsx). This wasn't seasonality because seasonality is an elderly effect and happens only near December. Also, if it was seasonal, it

would repeat at the same levels.



If you match up cohorts on deaths per week, y**ou find the cohorts diverge after the booster shots are given even though we are using fixed cohorts and following the SAME cohort longitudinally**. You cannot have a longitudinal analysis of a fixed cohort who are given a life-saving intervention have mortality that goes the wrong way (Source: KCOR_CMR_analysis):



Harm >> any benefit. KCOR shows that the sustained mortality rate increase was significantly larger than the time limited mortality rate differential during peak COVID→ harm > benefit. This is easiest to see by the value of KCOR(t) at the end of 2022.

The negative log-slope for elderly unvaccinated is normal and due to cohort heterogeneity. You must adjust for this to compare cohorts. This is a standard step in KCOR. No other epidemiological method does this. Yet, without that step, your comparisons are significantly wrong. Most epidemiologists are clueless about this.

4. My 3 cited papers and what they show:

   Qatar study: They did the most extreme 1:1 matching ever done in a study. They found two things: 1) you can't adjust away HVE using covariates and 2) despite a huge HVE bias reducing death, aHR after 6 months was >1.

   "In the first 6 months of follow-up in the two-dose analysis, the aHR was **0.35** (95% CI: 0.27–0.46); however, the combined analysis of all subsequent periods showed an aHR of **1.52** (95% CI: 1.19–1.94). In the first 6 months of follow-up in the three-dose analysis, the aHR was **0.31** (95% CI: 0.20–0.50); however, the combined analysis of all subsequent periods showed an aHR of **1.37** (95% CI: 1.02–1.85)."

   So even with a huge HVE advantage, the aHR went from extremely protective to >1 after 6 months showing vaccine harm, an incompensable HVE effect, and **confirming what KCOR found** about significant mortality increase by 6 months.

   Denmark study: They also did anal 1:1 matching and effectively said "we give up. HVE so big and hard to neutralize, we need RCTs. Traditional epi analysis methods don't work." Here is their Conclusion:

   Conclusion: Our findings indicate that observational studies of SARS-CoV-2 vaccine effectiveness may be **subject to substantial confounding**. Therefore, **randomized trials are essential** to establish vaccine efficacy after the emergence of new SARS-CoV-2 variants and the rollout of multiple booster vaccines.

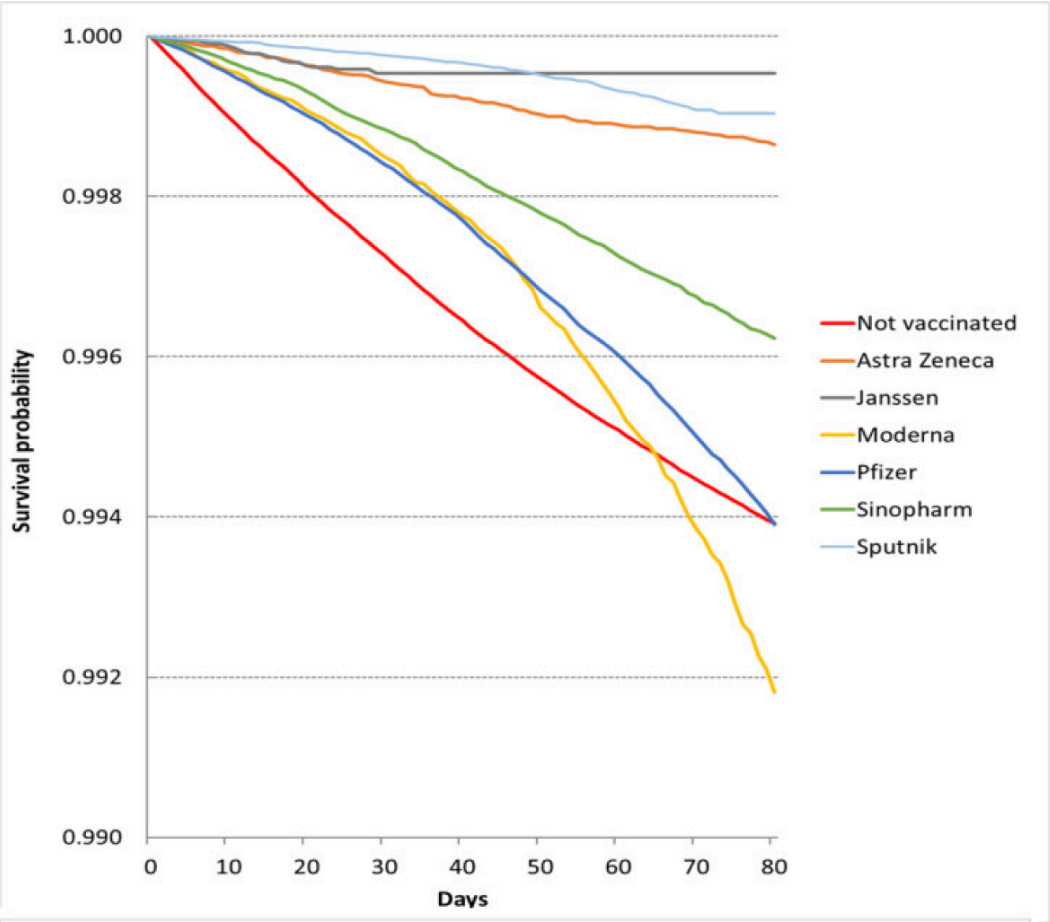
   But what they should have highlighted, but chose to avoid, is the negative control failed for the vaccinated cohort: 3.05 (95% CI: 2.24–4.14) for malignant melanoma. In short, vaccination negatively impacts your immune system so we have biologic plausibility here. This is a smoking gun of immune system harm.

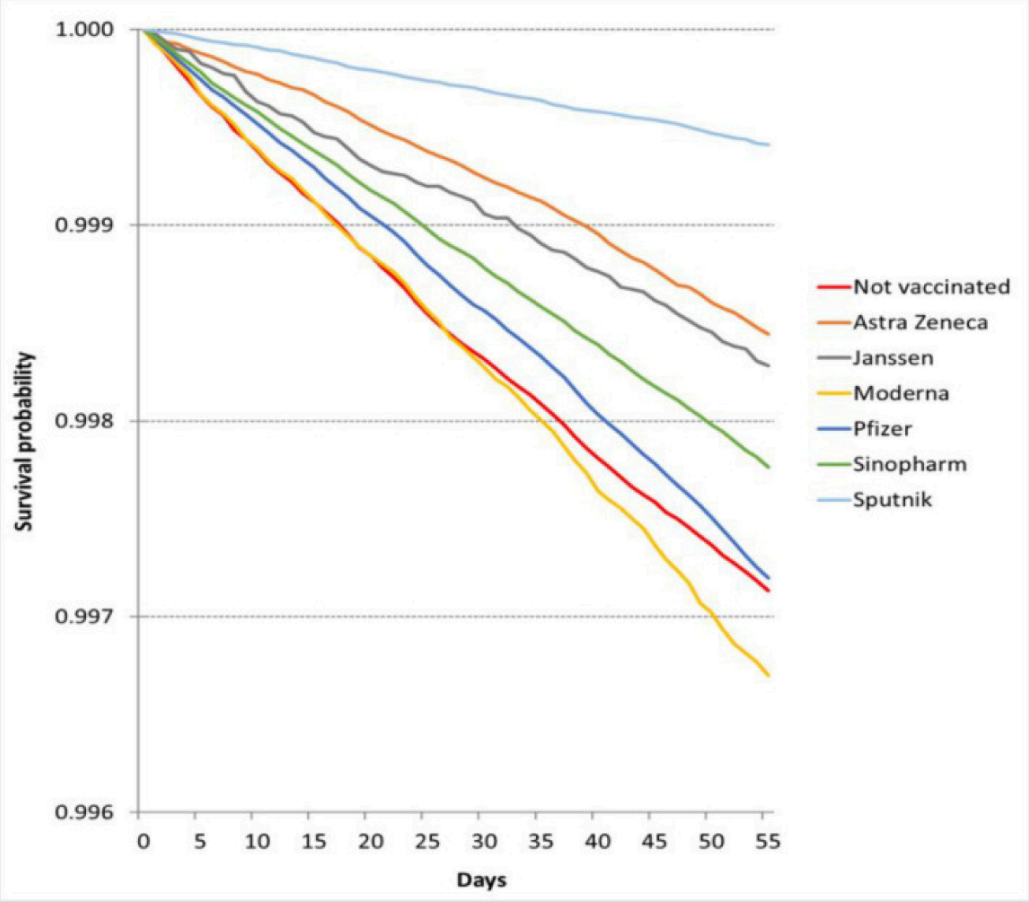
   When a study like this shows "favorable" noise in non-biological outcomes (like trauma deaths) but *unfavorable biological outcomes* (like cancer), anyone with intellectual honesty should admit that the *supposed safety reassurance is actually the opposite:* ***it's a huge red flag.***

   So this study confirms harms to the immune system and **also, like the Qatar study, throws all other COVID vaccine observational studies under the bus**

because you can't compensate for the HVE effect. This is why KCOR is so important because it works where other epi methods fail. It is the only method using slope normalization, for example. No other epi method does this; KCOR is unique to vaccinated mortality studies. KCOR is a general purpose epi method that is particularly adept at dealing with vaccine net mortality assessments.

Hungary study: Fig 1 and 2 are the smoking guns. These are KM plots. They reveal 3 huge red flags:


1.000
0.998
0.996
0.994
0.992
0.990
Survival probability
0
10
20
30
40
50
60
70
80
Days
Not vaccinated
Astra Zeneca
Janssen
Moderna
Pfizer
Sinopharm
Sputnik


1.000
0.999
0.998
0.997
0.996
Survival probability
0
5
10
15
20
25
30
35
40
45
50
55
Days
Not vaccinated
Astra Zeneca
Janssen
Moderna
Pfizer
Sinopharm
Sputnik

i. The curvature (instantaneous hazard value) of the unvaccinated cohort is constant over time. This validates the KCOR assumption of constant hazards (that can be slope adjusted). The vaccinated curves have opposite curvature (and higher than expected if no selection bias) exactly as expected.
ii. The Moderna cohort was younger and healthier than Pfizer. Yet Moderna had the worst survival of all cohorts. That can't happen with a safe vaccine, even if it had no protection. Moderna tracked Pfizer during COVID (Fig 1), but during non-COVID, Moderna started off significantly WORSE than Pfizer which is most likely because Moderna recipients had their mortality increased more than Pfizer.
iii. In Fig 1, Moderna tracked Pfizer at the start which means the cohorts were comparable. But by the end of the period, Modena had a huge negative divergence. This is a sign of vaccine harm.
iv. Moderna tracked the unvaxxed cohort in Fig 2, then diverged down. Sign of harm because there is nothing that can differentially impact Moderna cf Pfizer

So you need to use non-traditional methods to get an accurate assessment. Otherwise, it's all speculation because the HVE effect can swamp the signal. However, in all three of these papers, the vaccine mortality increase is so large, it exceeded the HVE protection:

v. **Qatar**: aHR>1 after 6 months
vi. **Denmark**: Malignant melanoma >3 IRR even though the cohorts were matched
vii. **Hungary**: The KM curves showed no difference in survival for Pfizer v. unvaxxed: same endpoint in both periods. Moderna clearly showed vaccine harm due to sudden divergence halfway through Fig. 2.

5. KCOR is the best method for data comparison because it matches the cohorts' mortality and log-slope. It shows large statistically significant net harm

   No mistakes in the method have been identified
   No superior method for cumulative risk/benefit analysis has been identified
   Graphs of harm v2 vs. v0. 1940 shows clear statistically significant harm. Harm was dose dependent, linearly proportional to dose. "Benefit" was independent of dose because it was caused by the cohort mortality/frailty differences.
   Vn vs Vn-1 are always elevated because the newly vaxxed increase mortality and then plateau. Less recent cohorts don't show a harm signal because the mortality elevation is time restricted.

6. KCOR on Czech validated using 2 different existing methods with similar curves (details in skirsch KCOR Github) so this was not a fluke.

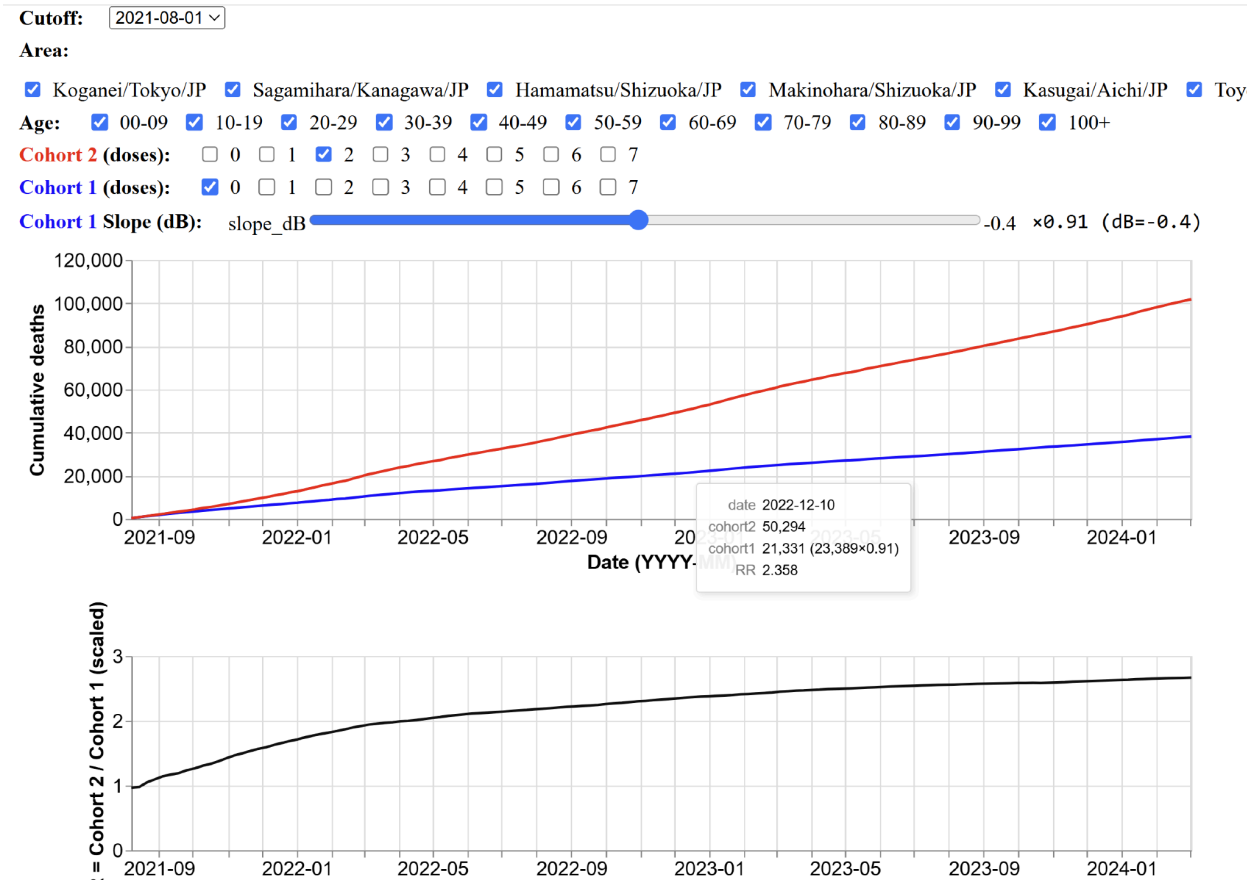
   DS-CMRR
   GML

7. Dynamic HVE cannot explain the mortality rise in recently vaccinated

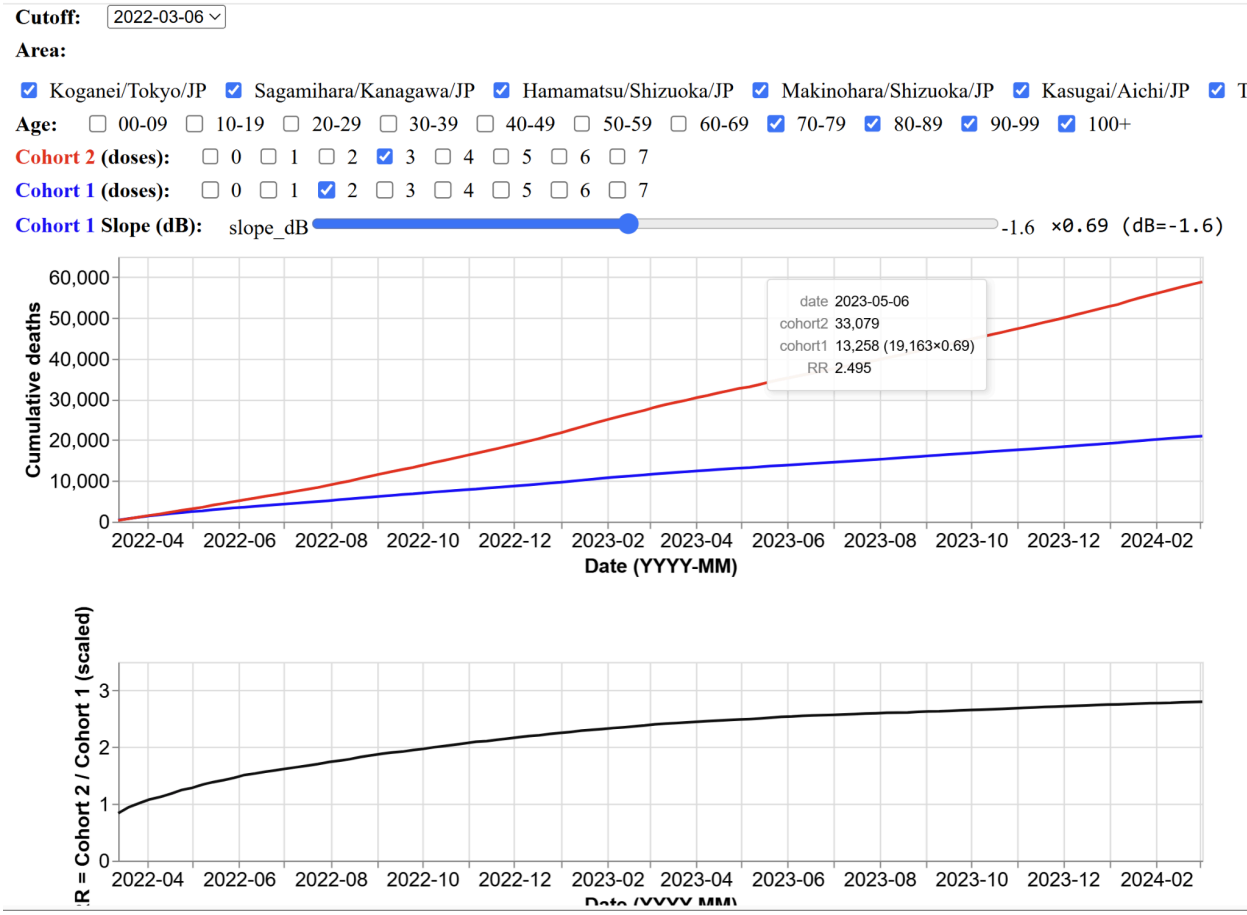
   See above

Also, the harm signal is stronger for younger people where the HVE is smaller.

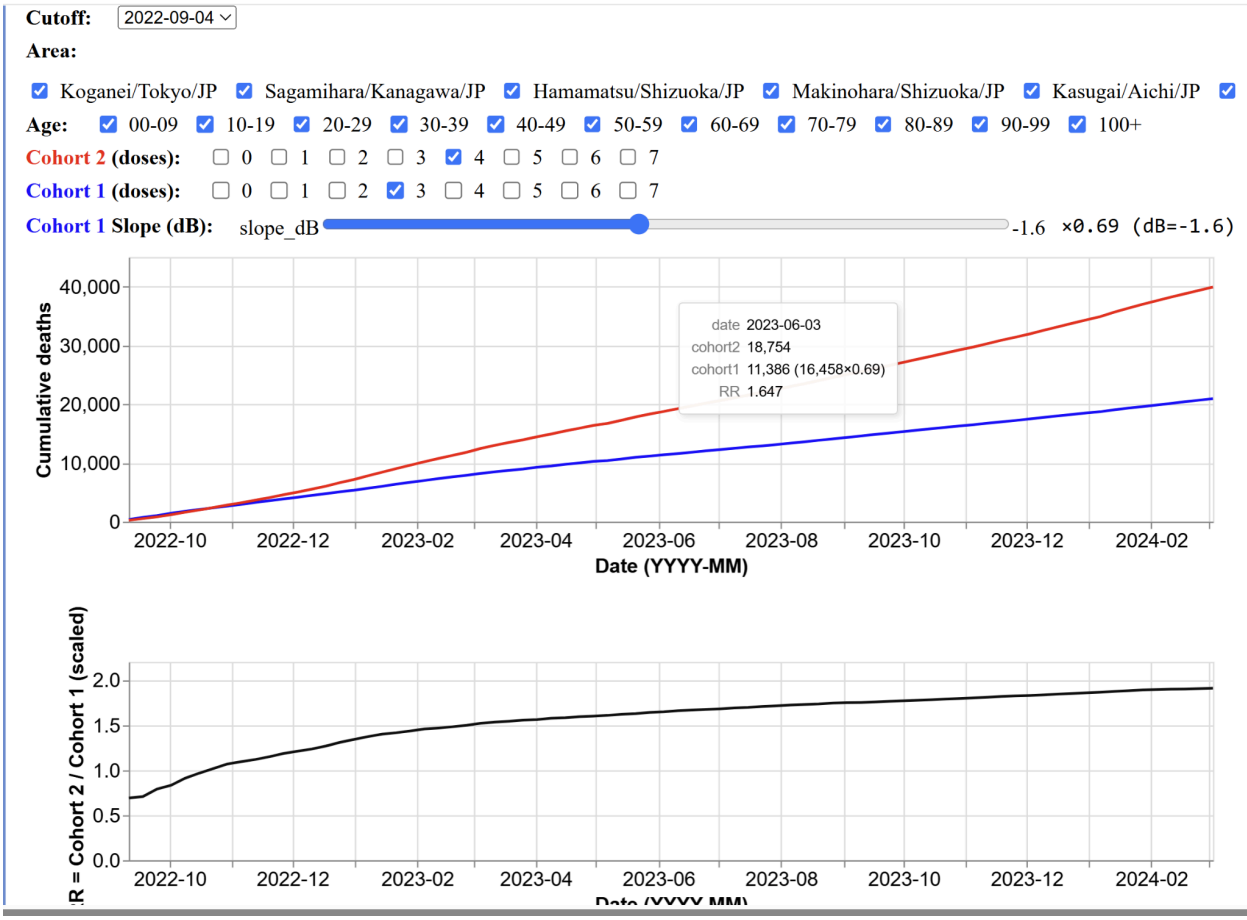
8. Easiest way to show net impact is longitudinal mortality divergence of the same cohort:
   See 1e above.
   Japan CMRR: [interactive site](#) shows divergence of latest shot vs. previous shot for all shots except the last 4 graphs showing same rise in latest dose, not those who avoid the dose.
9. When you compare people with similar mortality/frailty, the COVID benefit goes away.
   See 1a
10. 9 different methods on Czech support net harm. Can all of them be wrong? I couldn't find one favoring the "net benefit hypothesis."
11. Claims using time-varying ASMR charts are useless
   You must **mortality AND frailty match AND use FIXED cohorts to compare cohorts properly**; age and comorbidity matching is insufficient as we know from the Qatar paper
   Time varying cohorts creates too many confounders; so the time-varying ASMR charts that SW loves to show are totally misleading.
12. Japan results
   [Japan CMRR site](#)
   This is dose 2 vs. 0. Dose 2 mortality climbs and plateaus



Same effect for dose 3 vs. dose 2… the track and then the vaccinated die more.

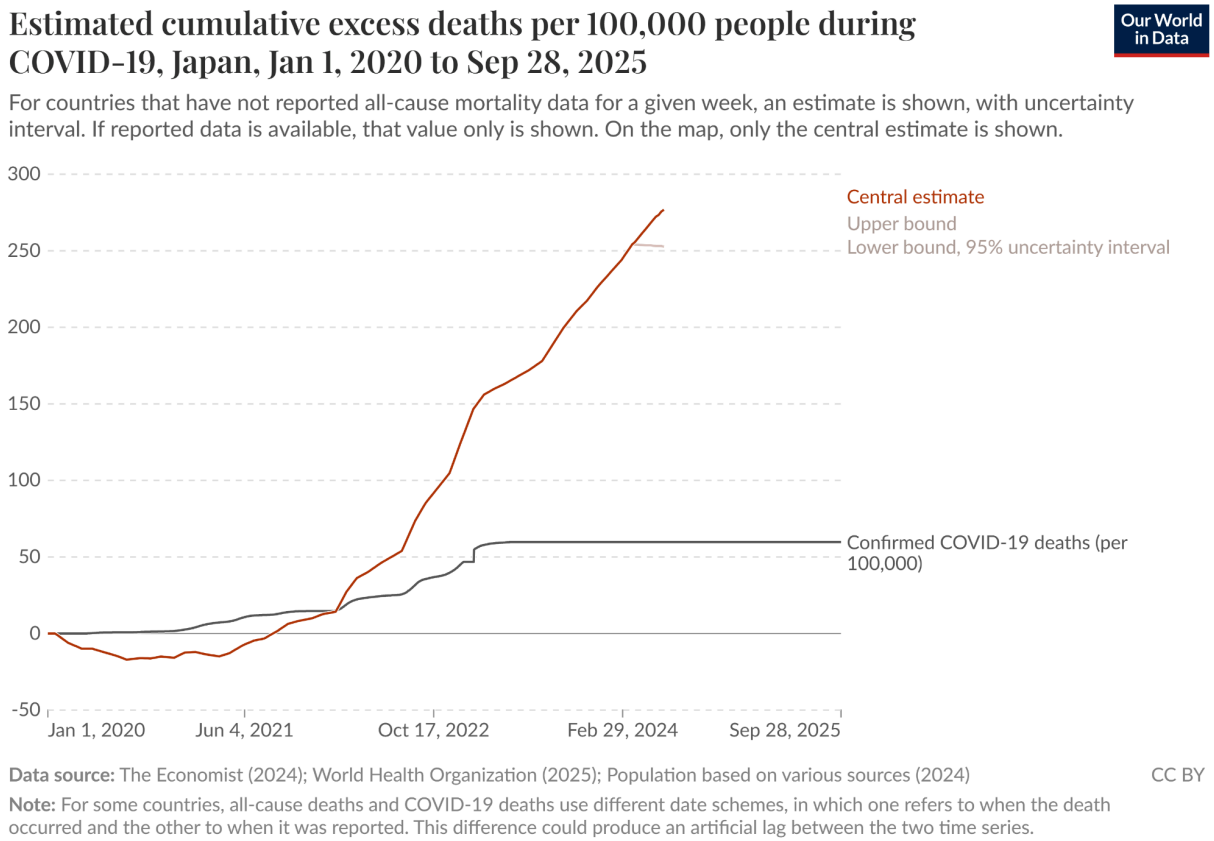


Same effect for dose 4 vs. dose 3… the curves track and then the vaccinated die more.



Not HVE because 99% took the shots so there are simply insufficient peope to absorb the deaths. Also, dose 3 v 0 rises, but 2 to 0 is flat; the age 60 cohort is the easiest to see.

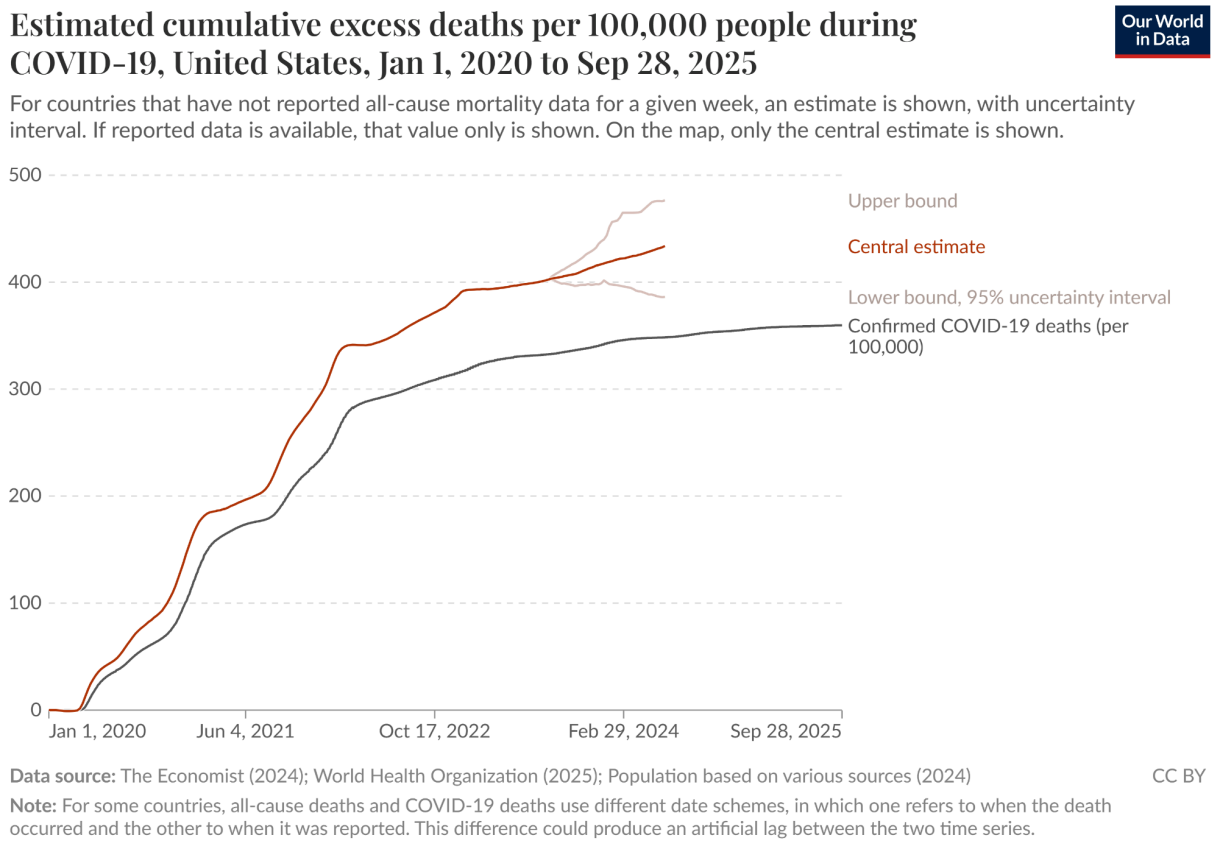
Japan was the highest vaccinated country on earth. OWID shows massive, **unprecedented excess deaths**. This is the confirmation that it wasn't some confounder causing these signals.



THIS IS THE KEY POINT

US results

US had higher excess mortality per 100K persons than Japan and Australia @400 per 100K people.



The “it’s HVE argument”

SW claims the rise in mortality is simply a return to baseline and that mortality is depressed after vaccination due to dynamic HVE.
This is a highly unlikely explanation for many reasons:

1. Dynamic HVE has a very short half life (around a week), so the effect diminishes rapidly
2. No support in any paper ever published ever
3. Everyone wanted the COVID shots
4. The US Medicare data shows a crystal clear 1 week half life for the COVID shot
5. The sheer magnitude of the numbers of people that would have had to decline the shots is more than the number of people who are about to die

It is easily falsified. We can use KCOR and we can also use the raw data.

However, for cohorts older than 60 years old, there is a third type of HVE that is noticeable which is caused by heterogeneity in the unvaccinated cohort which causes the log-slope of hazard(t) to be less than +8.5%/yr and often negative. This also impacts the vaccinated cohorts, but to a lesser extent since it is much larger. This effect can be easily seen in the KM curves above (upward curvature of the unvaccinated cohorts).

This effect is precisely why KCOR slope normalizes. No other epi method does this.

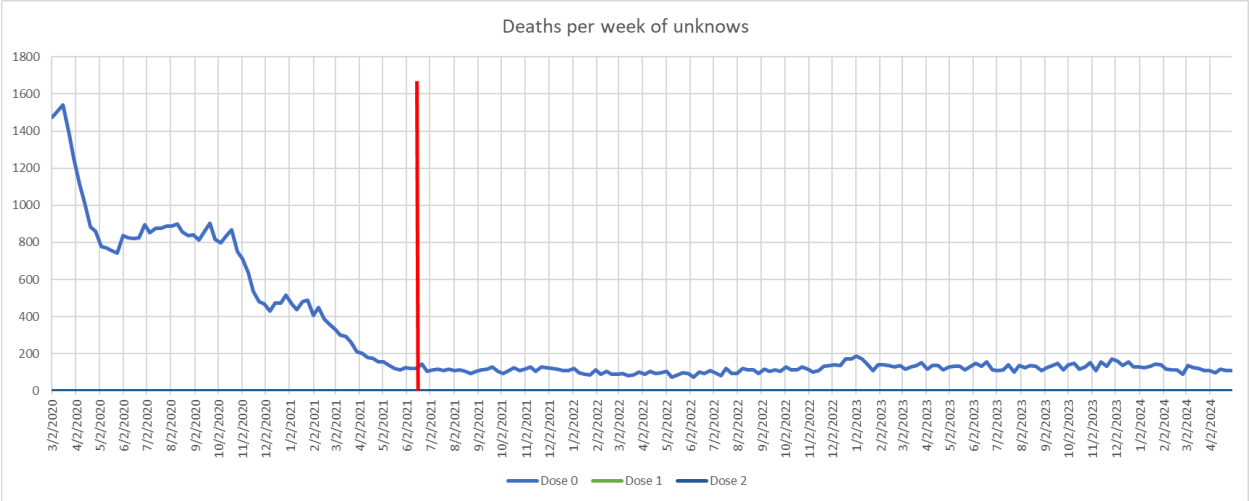
This is why KCOR is, to date, the best method to assess vaccine harm/benefit and nobody has been able to falsify it.

Appendix

Czech data very accurate

Two things to note:

1. Before mid 2021 (red line), there were more deaths of those with unknown birthdays. Ref: KCOR_CMR: unknown deaths over time.
2. After Jan 2024, deaths were not fully reported due to database prep cutoff date. Drops rapidly after April 2024.



Other Czech analysis methods

a. KCOR
b. Fixed cohorts (mortality matched): cum death divergence
c. CFR ratio during Delta, Omicron were comparable to mortality ratios of u/v during non-COVID
d. Time-series slope analysis (confirmed in 3 other countries) showing mortality rise tied to vaccination time
e. Variable cohorts: ACM ratios low v. high COVID (multiple narrow ages) showing increases are proportional to mortality differences
f. ACM relative increase: baseline (non-COVID) vs. full period: In 10 different age ranges, the relative ACM increases are higher for the vaccinated

g. Cum slope discontinuity analysis: ACM death, COVID cases, COVID deaths showed no discontinuity when the shots rolled out
h. Fixed cohorts: Frailty matched comparison show if mortality rates between vaccinated and unvaccinated track during non-COVID, they also track during COVID.
i. Fixed cohorts: CMR charts all show vaccinated mortality rises relative to unvaccinated cohort no matter how you pick the cohorts

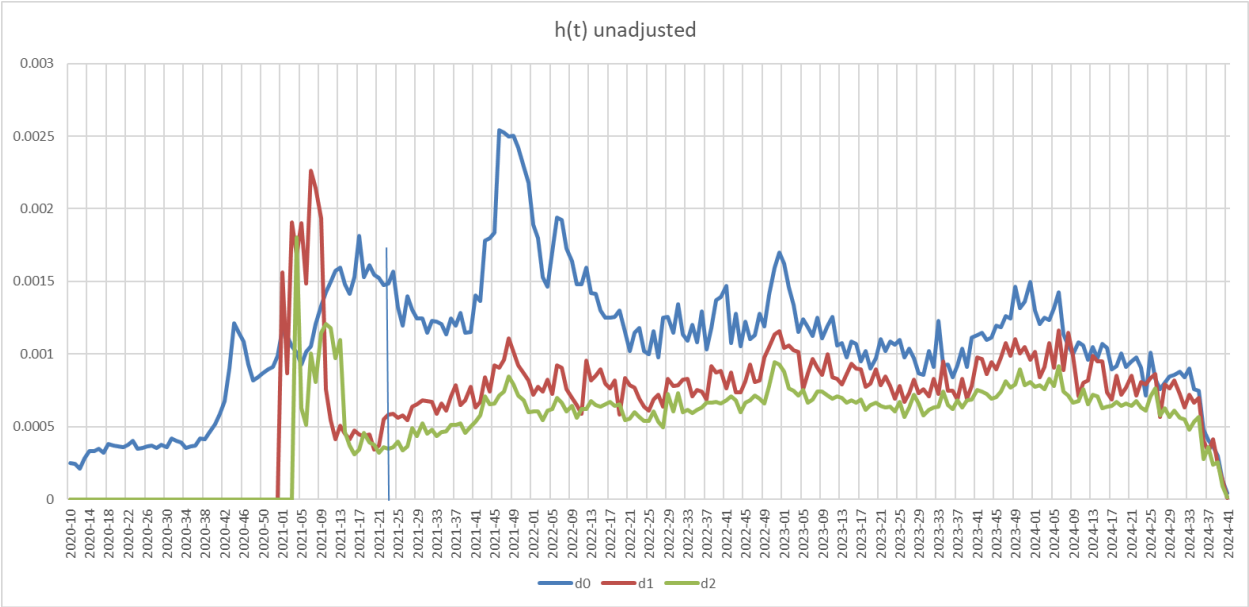
41. Detecting the COVID vaccine net mortality impact from observational studies is extremely challenging for a number of reasons:
    a. HVE effects must be measured accurately but HVE can only be measured after the intervention is given
    b. The COVID virus was a non-proportional mortality hazard (unlike most hazards)
    c. The COVID virus had different variants, each with a mortality rate that was not well characterized in control groups
    d. The vaccines were given at the same time there was a COVID wave
    e. COVID comes in waves that can have multiple peaks
    f. There was a tremendous amount of politicization, e.g., attributing cause of death to COVID rather than the vaccine
    g. The death effects were relatively small (10% mortality increases over a limited time period that can easily hide in seasonal/background noise)
    h. The causes of death were widely varied (not a single telltale cause of death)
    i. The death effects led to higher mortality than normal for a limited amount of time after the shots, but not a permanent mortality increase. Therefore, ascribing these deaths to the vaccine vs. background effects is difficult.
    j. People got vaccinated continuously over a long period of time so seeing a population mortality spike post-vaccination is harder to see because it could be misattributed if the study is not carefully done.
    k. Cumulative adjusted hazard curves can hide modest increases in mortality, e.g., a 2x mortality rate increase over 4 weeks is still a blip on the curve.
    l. The pull-forward effect can mask post-vaccine mortality harms because the shots were given during a COVID wave.
    m. The initial vaccination, boosters and second booster campaigns all overlapped COVID waves (see vaccine rollout tab of KCOR_CMR_analysis) which means teasing out the deaths is hard unless you use cohort data. You can't do it from population analysis.
    n. There were vax rollout during seasonal highs and/or COVID waves making the effects hard to pull apart without access to vaccination status of the dead

Baseline unadjusted h(t) grows faster in the vaccinated cohort than normal. Something is making the vaccinated die faster than they should be dying.

KCOR is simply confirming what we see in the data itself.

Something is impacting the h(t) of the vaccinated such that they are dying faster than normal. Can you help me find the cause?

Look at the h(t) unadjusted chart in the 2021_24 enroll 1940 tab of the KCOR_sample_computation spreadsheet.



I noticed the baseline h(t) (the troughs) were growing much faster than the normal 8.5%/yr rate, close to 2x higher (i.e., 95% CI: 16–19%/yr).

What could explain this if it wasn't the vaccine? These are fixed cohorts.

AI couldn't explain it if it wasn't caused by the vaccine Can you?